

# Respiratory Scenario 1

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| Scenario Set Up                         | <p>Equipment Needed:, O2, pulse Ox, stethoscope</p> <p>Proctor: You are a 35 year old male who is in tripod position and visibly anxious. Your wife called EMS and is on scene to provide details</p>                                                                                                                                                                                                                                                                                                                                                                                                      |
| Dispatch                                | Respond C3 to a house possible respiratory arrest. Neighbors are present and there is a lot of screaming.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                  |
| Scene Size Up                           | Pt found in tripod position in living room and is having difficulty breathing. Wife is present and visibly anxious                                                                                                                                                                                                                                                                                                                                                                                                                                                                                         |
| Pertinent Primary Assessment Findings   | <p>A - Airway clear and Patent<br/> B - Breathing is fast and labored<br/> C - Pale, cool and Diaphoretic</p> <p>AxO x4<br/> Person: John<br/> Place: His House<br/> Time: (Aligns with EMT's question)<br/> Event: "Can't Breathe"</p>                                                                                                                                                                                                                                                                                                                                                                    |
| Pertinent Secondary Assessment Findings | <p>S - Wheezing Lung Sounds<br/> A - No<br/> M - Albuterol<br/> P - Asthma<br/> L - Water at 3 PM, Urinated at 4:30 PM<br/> E - Was mowing the lawn outside when all of a sudden pt got a shortness of breath.</p> <p>P - Sitting Makes it better<br/> A - No associated chest pain.<br/> S - No sputum<br/> T - 3 Sentence Words<br/> E - Diminished</p> <p>O - Mowing lawn ^^<br/> P - Little relief after inhaler use, sitting upright helps, worse with activity.<br/> Q - Tight and Difficult<br/> R - No Pain, just "can't breath"<br/> S - "Can't breathe"<br/> T - 30 Minutes prior to arrival</p> |
| Vitals                                  | Initial Baseline Vitals: HR - 120, RR - 30/min, BP - 136/82, SpO2 - 90%                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                    |

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|                 | <p>Follow up if asked:<br/>         After Oxygen: HR - 108, RR - 24/min, BP - 134/80, SpO2 - 96%</p>                                                                                                                                                                  |
| Treatments      | <ul style="list-style-type: none"> <li>❖ Maintain Pt in a position of comfort (tripod/upright sitting)</li> <li>❖ Administer Oxygen</li> <li>❖ Assess lung sounds</li> <li>❖ Assist with Albuterol</li> </ul>                                                         |
| Key Points      | <ul style="list-style-type: none"> <li>- Airway is patent but breathing is inadequate</li> <li>- SpO2 of 90% = hypoxemia</li> </ul> <p>Should reassess every 5 minutes.</p>                                                                                           |
| Bonus Questions | <ol style="list-style-type: none"> <li>1) What is the difference between respiratory depression and distress?</li> <li>2) When would you use a BVM?</li> <li>3) Is it harder to Exhale or Inhale when your trachea closes up during an asthma attack? Why?</li> </ol> |

## Respiratory Scenario 2

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| Scenario Set Up | <p>Equipment Needed: Suction/OPA, BVM, O2, pulse Ox, AED, PPE</p> <p>Proctor: You are a 5 year old girl named Maya leaning forward with hands braced against your knees. You keep your mouth slightly open and are drooling. Mom is present and visibly anxious, but is able to provide history</p> |
| Dispatch        | <p>Respond C3 to a residence for a five year old girl with difficulty breathing with a high fever</p>                                                                                                                                                                                               |
| Scene Size Up   | <p>Pt. is seated in the living room in tripod position and drooling. Mom says “She woke up with a really bad sore throat and got much worse over the last few hours. She won’t lie down, and she keeps spitting because it’s too hard to swallow”</p>                                               |

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| Pertinent Primary Assessment Findings   | <p>A - Airway is full of secretions and child is unable to manage airway<br/> B - Breathing is fast and labored at 34/min<br/> C - Skin is Pale, warm, and diaphoretic; Pulse is 142 beats/min</p> <p>A and O<sub>x4</sub><br/> P - Maya is recognized and responds to her name<br/> P - Maya says “im at home”<br/> T - “Nighttime”<br/> E - “It hurts to swallow”</p>                                                                                                                                                                                                |
| Pertinent Secondary Assessment Findings | <p>S - Stridor heard on inspiration, fever, drooling, sore throat<br/> A - No known allergies<br/> M - no medications<br/> P - No pertinent medical history<br/> L - Small amount of juice 6 hours ago<br/> E - Fever and Sore throat began earlier today</p> <p>O - Fever started earlier today, and drooling began about 2 hours ago<br/> P - Sitting up right helps, and lying down makes the patient feel worse<br/> Q - Sore throat<br/> R - Only the neck region<br/> S - severe throat pain<br/> T - Child has been rapidly worsening over the past 2 hours</p> |
| Vitals                                  | RR - 34, HR - 142, BP - 104/96, SPO2 - 93%, Temperature - 103.1 F                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                      |
| Treatments                              | <ul style="list-style-type: none"> <li>• Keep in position of comfort</li> <li>• Administer blow-by oxygen</li> </ul>                                                                                                                                                                                                                                                                                                                                                                                                                                                   |
| Key Points                              | <ul style="list-style-type: none"> <li>• Avoid examining the oral cavity or placing anything in it</li> <li>• Should Reassess every 5 minutes</li> <li>• Epiglottitis features the 3Ds - Drooling, Dysphagia, and Distress</li> </ul>                                                                                                                                                                                                                                                                                                                                  |
| Bonus Questions                         | <ul style="list-style-type: none"> <li>• Why is the main diagnosis suspected epiglottitis and not croup? (seal bark cough)</li> <li>• Why is keeping the child upright and close to the parent vital for airway management?</li> </ul>                                                                                                                                                                                                                                                                                                                                 |

## Respiratory Scenario 3

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| Scenario Set Up                         | <p>Equipment Needed: O<sub>2</sub>, pulse Ox, stethoscope</p> <p>Proctor: You are a 22-yo male who is sitting upright and appears anxious. You are complaining of sudden chest pain and difficulty breathing. Your roommate calls EMS</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                |  |
| Dispatch                                | Respond C3 to a college apartment for a possible respiratory emergency. Patient conscious and having difficulty breathing.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                               |  |
| Scene Size Up                           | Pt found sitting upright on the couch, anxious and holding the right side of his chest. Roommate is present.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                             |  |
| Pertinent Primary Assessment Findings   | <p>A - Airway clear and patent<br/> B - Breathing is fast and labored<br/> C - Pale, cool, and anxious</p> <p>AxO x4<br/> Person: Nick<br/> Place: His Apartment<br/> Time: (Aligns with EMT's question)<br/> Event: "Hurts to Breathe"</p>                                                                                                                                                                                                                                                                                                                                                                                                                                              |  |
| Pertinent Secondary Assessment Findings | <p>S - Wheezing Lung Sounds<br/> A - No<br/> M - None<br/> P - No Significant History<br/> L - Lunch at 6 PM, Urinated at 4:30 PM<br/> E - "Was sitting on my couch when all of a sudden the right side of my chest started to hurt and it was hard to breathe."</p> <p>O - Sitting on Couch ^^<br/> P - Worse with deep breaths, sitting upright helps.<br/> Q - Sharp, stabbing chest pain<br/> R - No Radiation<br/> S - 8/10 Pain<br/> T - 20 Minutes prior to arrival</p> <p>Lung Sounds: Left side is WNL, Right side is diminished.</p> <p>P - Sitting Makes it better<br/> A - "Stabbing" pain upon inhale.<br/> S - No sputum<br/> T - 3 Sentence Words<br/> E - Diminished</p> |  |

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| Vitals          | <p>Initial Baseline:<br/>HR - 118, RR - 30/min, BP - 132/84, SpO2 - 91%</p> <p>Follow Up After Oxygen:<br/>HR - 108, RR - 24/min, BP - 134/82, SpO2 - 95%</p>           |
| Treatments      | <p>Maintain position in a position of comfort</p> <p>Administer Oxygen</p> <p>Assess lung sounds frequently</p> <p>Rapid transport</p> <p>Reassess every 5 minutes.</p> |
| Key Points      | <p>Watch for signs of a tension pneumothorax</p> <p>Do not attempt to decompress the chest</p> <p>Pay attention to any decreased/absent breath sounds on one side.</p>  |
| Bonus Questions | <p>What is the difference between a tension, spontaneous and hemopneumothorax?</p>                                                                                      |